Mental Health Coverage: How Has Policy Impacted Mental Health Rates Compared to Physical Health?

Insurance is fundamentally built on a single promise: providing a financial safety net against life's unforeseen emergencies. People make the tedious monthly payments in the hope that if one day their house floods, they crash their car, or they suddenly need surgery, those monthly payments will step up and prevent the emergency from burning a hole in their wallets, providing reassurance during an emergency. Despite the broad coverage insurance provides across many categories, there is debate as to whether this reassurance is limited to physical health or if it should extend to mental health as well.

According to HealthCare.gov, the Affordable Care Act ensures that most plans cover behavioral health treatment, mental health inpatient services, and substance use disorder treatment. In addition, there are restrictions on how insurers can cover preexisting mental health conditions. Under the Affordable Care Act, Marketplace health insurance plans cannot charge a person more or deny them coverage because of a preexisting condition. "Parity" protections mandate that services tailored to mental health and substance abuse cannot be more restrictive than coverage for physical health treatments.

Barriers to Mental Health Treatment

While the Affordable Care Act promotes equal mental health coverage across platforms, the actual implementation of these policies makes it much harder to navigate the medical landscape.

According to the American Psychological Association, "On January 13, 2026, the Substance Abuse and Mental Health Services Administration (SAMHSA) terminated hundreds of grants supporting mental health and substance use disorder services, totaling approximately $2 billion."

Protests from mental health advocates resulted in the "funding [being] reinstated on the evening of January 14," the American Psychological Association stated.

Despite the restoration, the original decision to terminate the funding demonstrates how mental health is often cast aside and perceived as "less important" when compared to medical and surgical treatments.

While the public was able to successfully avoid funding cuts from SAMHSA, other policies under the administration have a more tangible impact. In May 2025, the administration stated that it would not enforce the Biden-era mental-health parity regulations (American Psychological Association). The lack of enforcement of these regulations means that requirements providing equal coverage for both physical and mental health will be weakened. As a result, mental health treatments run the risk of receiving less coverage than physical health.

Additionally, the “Big Beautiful Bill Act” cut Medicaid funding by 15%. The nonpartisan Congressional Budget Office estimates that this 15% cut, or $1 trillion over a period of 10 years, will cause 11.8 million people to lose their Medicaid health insurance coverage. The Department of Health and Human Services has also proposed restructuring federal mental health agencies by reducing funding and dissolving SAMHSA (American Psychological Association). Given that over 4.4 million out of the 24.3 million Marketplace enrollees are estimated to have at least one mental health diagnosis, cutting these mental health agencies is projected to cause an additional 8.2 million people to lose their Marketplace coverage and become uninsured in 2034 (Panchal et al.). Taking away mental health coverage from people with pre-existing conditions would make it more difficult for people to get the help they need, potentially exacerbating their struggles.

What Happens Now?

Under the new administration, mental health is being treated as a luxury rather than a basic right. The prospect of millions of people being uninsured in 2034 not only represents an economic weakness but also makes navigating personal struggles much harder.

The Affordable Care Act includes coverage for behavioral health as an essential health benefit, not an add-on. As a result, many insurance plans are legally required to cover mental health and substance use disorders at the same level as physical health conditions. If the administration continues its focus on dismantling mental health agencies, then insurance will no longer serve as a reassuring shield against the high costs of psychiatric care. Instead, it will regress into a system that treats mental health as a personal liability, erasing decades of progress towards healthcare equity.

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